THE HOSPITAL AND HEALTHSYSTEM ASSOCIATION OF PENNSYLVANIA

340B: Protecting access to care in Pennsylvania

ADVOCACY BRIEFING — MARCH 2026

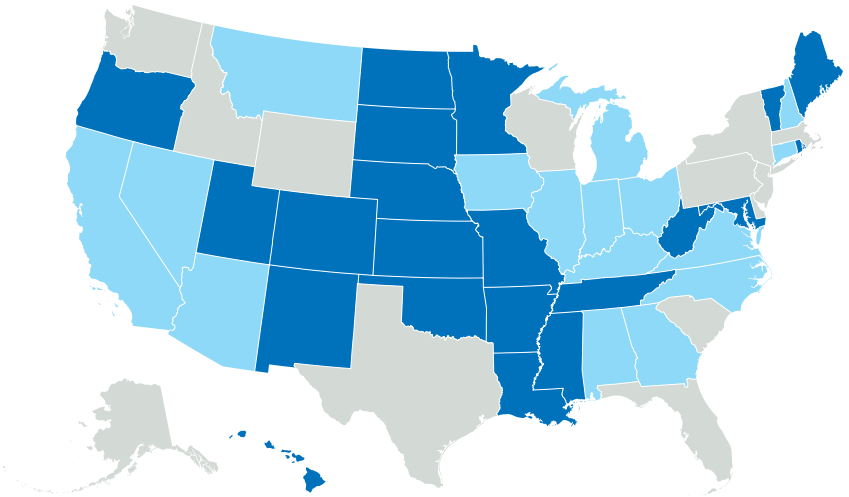
REPORTED COMMUNITY BENEFIT (2024)
$7.95B
Reinvested in patient care and community services

PA HOSPITALS IN 340B
72
30% of Pennsylvania's 235 hospitals

STATES WITH CONTRACT PHARMACY PROTECTION
21
29 states still lack protection — including PA

SHARE OF U.S. DRUG MARKET
7%
Small program, outsized impact for patients

CONTRACT PHARMACY PROTECTION BY STATE (U.S.)



Contract pharmacy protection enacted | PBM regulation only (no contract pharmacy protection) | No enacted contract pharmacy protection

WHAT HAP IS ASKING LAWMAKERS TO DO

Protect the 340B discount
Keep hospital access to the outpatient drug prices Congress set for charity care and community benefit.

Defend contract pharmacy partnerships
Patients fill prescriptions through local pharmacies — not one distant site selected by manufacturers.

Oppose rules that shrink safety-net access
Rural and underserved communities keep an affordable path to medications hospitals are required to support.

HRSA PROGRAM INTEGRITY FY 2024

Covered entity audits **179** vs. Manufacturer audits **5**

36× more hospital audits than manufacturer audits
Hospitals and manufacturers should face the same level of federal scrutiny. The current imbalance is an equity argument.

WHAT MANUFACTURERS ARGUE — AND THE RESPONSE

MANUFACTURER CLAIM	HAP RESPONSE	EVIDENCE
340B creates duplicate discounts.	**PA DHS runs the 340B Drug Exclusion List preventing this. Federal law (42 U.S.C. §256b) already prohibits duplicate discounts.**	PA DHS Exclusion List; HRSA program integrity rules
Hospitals use 340B savings for profit.	**$7.95B in reported community benefit (2024) funds free prescriptions, cancer screening, dental care, and rural services.**	340B Health 2024 community benefit report
Contract pharmacy networks are unaudited.	**HRSA conducted 179 covered-entity audits vs. only 5 manufacturer audits in FY 2024.**	HRSA Program Integrity FY 2024

STATE CONTRACT PHARMACY PROTECTION STATUS

STATE	CONTRACT PHARMACY	PBM LAW	YEAR	NOTES
Alabama	No	**Yes**	2021	
Alaska	No	No	—	
Arizona	No	**Yes**	2022	
Arkansas	**Yes**	**Yes**	2021	First to enact; upheld in court.
California	No	**Yes**	2023	
Colorado	**Yes**	**Yes**	2022	Contract pharmacy 2025.
Connecticut	No	**Yes**	2023	
Delaware	No	No	—	
Florida	No	No	—	
Georgia	No	**Yes**	2020	
Hawaii	**Yes**	No	2025	Reporting required.
Idaho	No	No	—	
Illinois	No	**Yes**	2022	
Indiana	No	**Yes**	2021	
Iowa	No	**Yes**	2023	
Kansas	**Yes**	No	2024	
Kentucky	No	**Yes**	2020	
Louisiana	**Yes**	**Yes**	2023	Upheld in court.
Maine	**Yes**	No	2025	Hybrid 2025.
Maryland	**Yes**	No	2024	
Massachusetts	No	No	—	
Michigan	No	**Yes**	2022	
Minnesota	**Yes**	**Yes**	2019	Upheld in court.
Mississippi	**Yes**	**Yes**	2024	
Missouri	**Yes**	No	2024	Upheld in court.
Montana	No	**Yes**	2021	
Nebraska	**Yes**	**Yes**	2022	
Nevada	No	**Yes**	2023	
New Hampshire	No	**Yes**	2024	
New Jersey	No	No	—	
New Mexico	**Yes**	**Yes**	2023	
New York	No	No	—	
North Carolina	No	**Yes**	2021	
North Dakota	**Yes**	**Yes**	2021	
Ohio	No	**Yes**	2021	Hybrid 2025.
Oklahoma	**Yes**	No	2025	
Oregon	**Yes**	**Yes**	2025	
Pennsylvania	No	No	—	In progress.
Rhode Island	**Yes**	**Yes**	2025	Upheld in court.
South Carolina	No	No	—	
South Dakota	**Yes**	**Yes**	2024	

STATE	CONTRACT PHARMACY	PBM LAW	YEAR	NOTES
Tennessee	**Yes**	**Yes**	2021	Upheld in court.
Texas	No	No	—	
Utah	**Yes**	**Yes**	2020	
Vermont	**Yes**	**Yes**	2021	Hybrid 2025.
Virginia	No	**Yes**	2021	Governor vetoed protection.
Washington	No	No	—	
West Virginia	**Yes**	**Yes**	2024	
Wisconsin	No	No	—	
Wyoming	No	No	—	

Sources: MultiState · ASHP · America's Essential Hospitals (state law) · 340B Health · AHA (community benefit) · HRSA Program Integrity FY 2024 (audit counts) · 42 U.S.C. §256b (federal statutory basis)

Limitations: State law counts change as legislatures meet. Community benefit totals are self-reported aggregates, not independently audited.